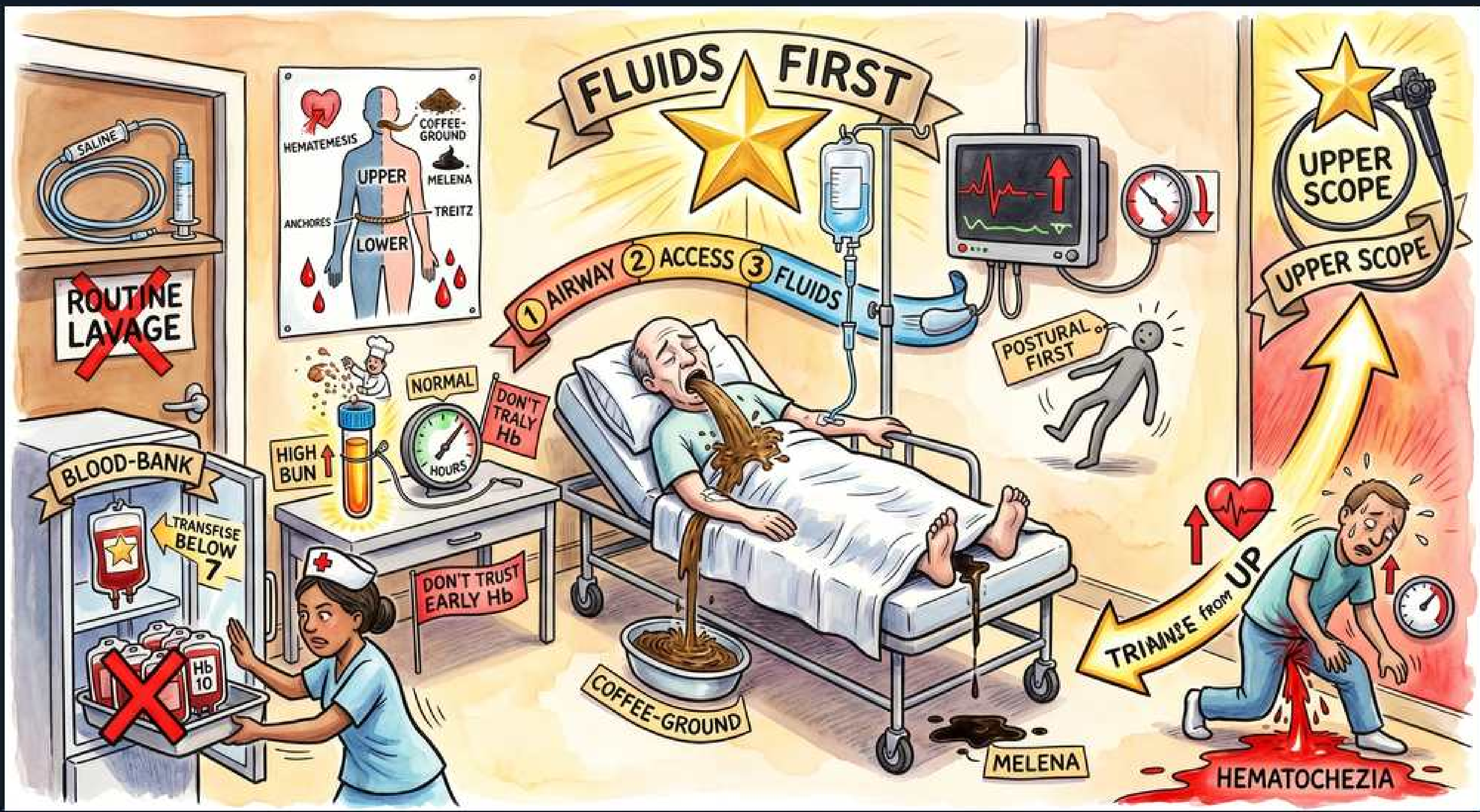
# GI Bleed — Triage & Localization



### 1. FLUIDS FIRST (gold star)

**WHAT YOU SEE**
Banner 'FLUIDS FIRST' with a gold star and an IV pole above the patient bed.

**WHAT IT ENCODES**
Resuscitate BEFORE you localize: ABC, two large-bore IVs (16-18G), crystalloid bolus. Hemodynamic stabilization always precedes diagnostic workup in GI bleed. Massive transfusion / blood products if shock. The sequence airway->access->fluids is the gold-star answer on any GI bleed stem.

### 2. Airway-Access-Fluids 1-2-3

**WHAT YOU SEE**
Numbered ribbon '1 AIRWAY 2 ACCESS 3 FLUIDS' across the patient.

**WHAT IT ENCODES**
Ordered priorities: 1) airway (protect against aspiration of blood/vomit, intubate if obtunded or massive hematemesis), 2) two large-bore IV access, 3) volume resuscitation with crystalloid then blood. This A-B-C order is the reflex first step before endoscopy.

### 3. Hematemesis/coffee-ground = UPPER

**WHAT YOU SEE**
Body diagram poster: 'HEMATEMESIS / COFFEE-GROUND / UPPER' marked near top of GI tract.

**WHAT IT ENCODES**
Hematemesis, coffee-ground emesis and melena = UPPER GI bleed, proximal to the ligament of Treitz. Coffee-ground = blood altered by gastric acid. Melena (black tarry stool) needs ~50 mL of blood and hours of gut transit to form.

### 4. Ligament of Treitz / Lower

**WHAT YOU SEE**
Poster lower half labeled 'LOWER ... TREITZ' on the body diagram.

**WHAT IT ENCODES**
The ligament of Treitz is the anatomic divide: bleeding proximal = upper, distal = lower. Hematochezia (bright red blood per rectum) usually means a LOWER source, but a brisk upper bleed can also present as hematochezia.

### 5. Coffee-ground emesis

**WHAT YOU SEE**
Pool of dark coffee-ground vomit labeled 'COFFEE-GROUND' beside the bed.

**WHAT IT ENCODES**
Dark granular vomit = blood oxidized by gastric acid into hematin; signals an upper source that is not actively brisk. Contrast with bright red hematemesis = active brisk upper bleed.

### 6. Melena

**WHAT YOU SEE**
Black tarry stool on the floor labeled 'MELENA'.

**WHAT IT ENCODES**
Black, tarry, foul stool = digested blood, almost always an UPPER source (occasionally small bowel / right colon). Requires GI transit time, so it lags the bleed. Not the same as melanotic drugs (iron, bismuth) which are non-tarry.

### 7. High BUN

**WHAT YOU SEE**
Beaker labeled 'HIGH BUN' bubbling on the side table.

**WHAT IT ENCODES**
Elevated BUN:creatinine ratio (>20-30:1) suggests an UPPER GI bleed: intraluminal blood is digested and absorbed as urea, plus prerenal hypoperfusion. A rising BUN with bloody stool points the source upward.

### 8. Postural / orthostatic vitals

**WHAT YOU SEE**
Gray standing figure labeled 'POSTURAL FIRST' beside the monitor.

**WHAT IT ENCODES**
Check orthostatic (postural) vitals: a drop in SBP >=20 or rise in HR >=30 on standing indicates significant volume loss (>15% blood volume) even when supine vitals look normal. Tachycardia is an early sign; hypotension is late.

### 9. Don't trust early Hb

**WHAT YOU SEE**
Banner 'DON'T TRUST EARLY Hb' near a gauge reading 'NORMAL' with 'DON'T HALVE Hb / HOURS'.

**WHAT IT ENCODES**
Initial Hb can be falsely NORMAL in acute bleed because plasma and red cells are lost proportionally; it equilibrates only over 24-72h as fluid shifts in. Don't recheck in 1-2h expecting truth, and never use a normal early Hb to exclude a significant bleed.

**BOARD TRAP**
'normal early Hb excludes significant bleed' -> wrong; Hb equilibrates late (24-72h), so an early normal value is unreliable.

### 10. Transfuse below 7 (restrictive)

**WHAT YOU SEE**
Blood-bank fridge banner 'TRANSFUSE BELOW 7' with bags of blood.

**WHAT IT ENCODES**
Restrictive transfusion threshold: transfuse PRBC for Hb <7 g/dL (<8 if known cardiovascular disease / active ischemia). Restrictive strategy lowers mortality and rebleeding versus liberal. Target ~7-9, not 10.

### 11. Transfuse to Hb 10 (RED X)

**WHAT YOU SEE**

Blood bag labeled 'Hb 10' with a big red X over it.

**WHAT IT ENCODES**

Liberal transfusion to Hb 10 is the wrong answer; correct restrictive target is <7 (<8 if cardiac). Over-transfusion raises portal pressure (worse in varices) and increases rebleeding and death.

**BOARD TRAP**

'transfuse to Hb 10' -> wrong; restrictive threshold is <7 (<8 with cardiac disease).

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### 12. Routine NG lavage (RED X)

**WHAT YOU SEE**

Saline NG setup on the door labeled 'ROUTINE LAVAGE' with a red X.

**WHAT IT ENCODES**

Routine nasogastric lavage is NOT recommended; it does not improve outcomes, does not reliably localize, and delays care. Endoscopy is the diagnostic and therapeutic test of choice. Erythromycin prokinetic before EGD is preferred over lavage to clear the stomach.

**BOARD TRAP**

'routine NG lavage to confirm/localize the bleed' -> wrong; it changes no outcomes and is not needed.

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### 13. Hematochezia = triage from UP

**WHAT YOU SEE**

Patient passing bright red blood labeled 'HEMATOCHEZIA' with arrow 'TRIAGE FROM UP'.

**WHAT IT ENCODES**

Bright red blood per rectum is usually lower GI, BUT ~10-15% of brisk upper bleeds present as hematochezia. If the patient is unstable, scope the TOP first to exclude a massive upper source before committing to colonoscopy.

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### 14. Upper scope arrow (gold star)

**WHAT YOU SEE**

Gold star with endoscope and 'UPPER SCOPE' banner, arrow rising from hematochezia patient.

**WHAT IT ENCODES**

When unstable hematochezia raises concern for an upper source, EGD first. EGD is also first-line for any suspected upper bleed; perform within 24h of resuscitation (urgently if unstable). This sets up the lower-pathway scene.

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